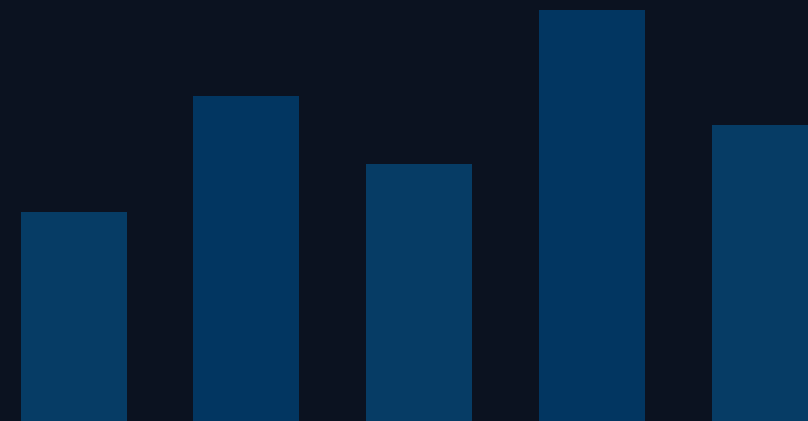


CMS Medicare Program Analytics

Enrollment, claims and utilization, Part B cost, plan quality, and provider enrollment in a single view



70.5M

BENEFICIARIES

\$377.3B

FFS PAYMENTS

3.7 / 5

AVG STAR RATING

2.98M

PROVIDER ENROLLMENT
RECORDS

76.8%

PART B PAYMENT RATE

The Bottom Line

One page for decision-makers: who Medicare covers, what it spends, how Part B costs break down, how plans perform, and how the enrolled provider network is composed.

70.5M

BENEFICIARIES

51.2% in MA & Other plans

\$377.3B

FFS PAYMENTS

\$12,553 per beneficiary

3.7 / 5

AVG STAR RATING

44.4% of contracts at 4+ stars

2.98M

**PROVIDER ENROLLMENT
RECORDS**

2.55M individual-practitioner enrollments

What the data says

- 01 Coverage has tipped to MA & Other plans**
MA and other plans now cover 36.1M of 70.5M beneficiaries — 51.2% — after crossing above Original Medicare.
- 02 FFS cost is rising per person**
Standardized payment per beneficiary climbed from \$9,191 in 2014 to \$12,553 in 2024.
- 03 Quality has plateaued below 4 stars**
The average contract has held near 3.7 of 5 since 2024, and only 44.4% reach 4+ stars.
- 04 Nurse practitioners lead primary provider types**
Nurse practitioners are the largest primary provider type at 359,719 unique provider entities within 2.98M enrollment records.
- 05 Part B spending is climbing**
Part B payments grew from \$115.9B in 2022 to \$131.7B in 2024, with the payment rate steady near 77%.

Where to focus

- 1 Plan for an MA-majority program**
Half of beneficiaries are in MA & Other plans — quality and risk levers outweigh FFS pricing.
- 2 Bend the per-beneficiary curve**
Hospital inpatient is 28.0% of FFS payments — the largest single block.
- 3 Close the four-star gap**
Contracts clustered at 3.5 stars are the clearest path to bonus-eligible performance.

What This Analysis Covers

Scope

Five CMS data domains — enrollment and demographics, FFS claims and utilization, Part B utilization and cost, MA plan quality, and provider enrollment.

Population

All Medicare beneficiaries nationally, plus rated MA contracts and the enrolled fee-for-service provider network.

Primary Question

Who does Medicare cover, what does it pay for, how do Part B services and costs break down by specialty, how well do plans perform, and how is the enrolled provider network composed?

Source

CMS Monthly Enrollment (May 2026), Geographic Variation (2014–2024), MA & Part D Star Ratings (2024–2026), Provider Enrollment PPEF (2026 Q3), and Part B PBAR (CY 2022–2024).

5

Data domains

70.5M

Beneficiaries covered

516

Rated MA contracts

2.98M

Provider enrollment records

Five Domains at a Glance

The five datasets in one line — 70.5M beneficiaries · \$377.3B FFS payments · \$131.7B Part B paid · 3.7 avg stars · 2.98M provider enrollment records.

01

Enrollment & Beneficiaries

70.5M covered, 51.2% of them in Medicare Advantage & Other plans, on a base weighted toward ages 65–74 (48.4%).

02

Claims & Utilization

\$377.3B of FFS payments in 2024 — \$12,553 per beneficiary, led by hospital inpatient at 28.0%.

03

Part B Utilization & Cost

\$131.7B paid on 5.96B allowed services in 2024 — \$22.08 per service at a 76.8% payment rate.

04

Quality & Star Ratings

516 rated contracts averaging 3.7 of 5; 44.4% reach 4+ stars and just 22 hit five.

05

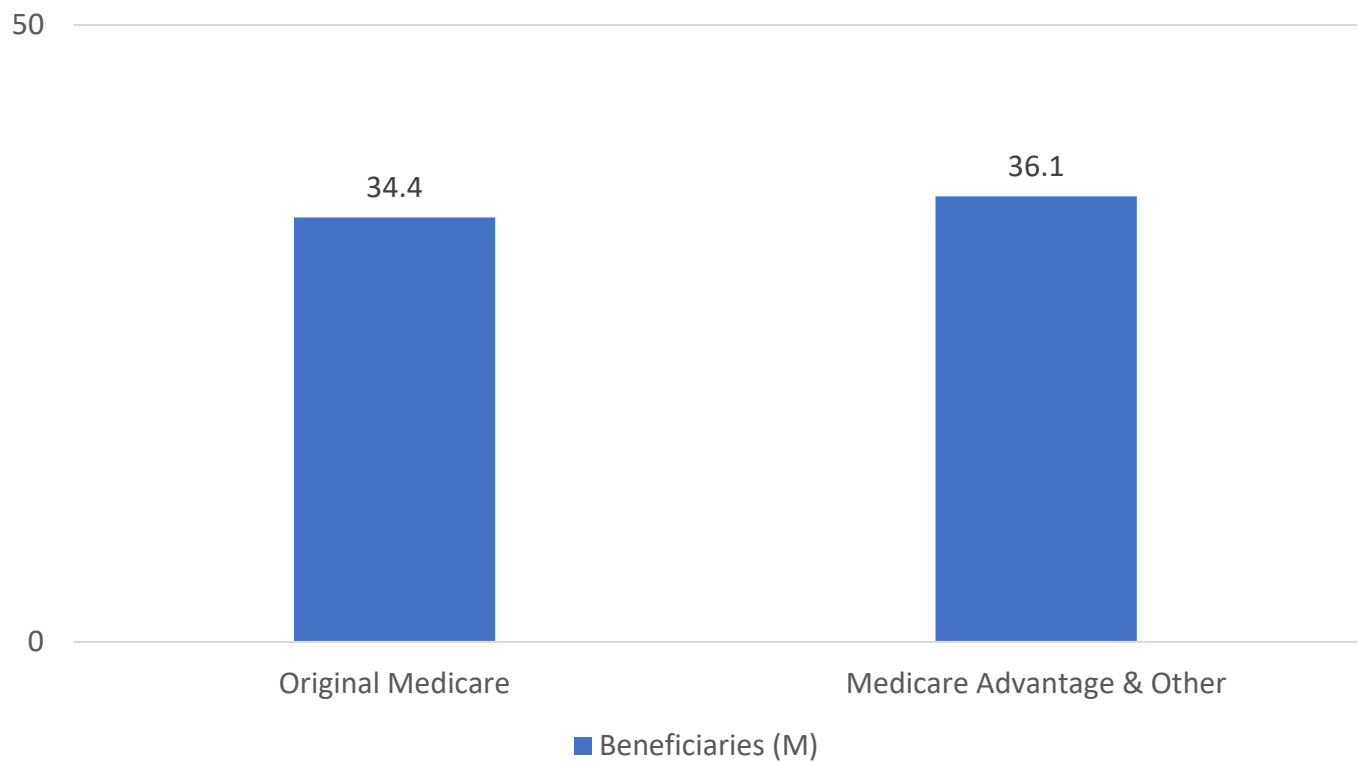
Provider Network

2.98M enrollment records — 2.55M individual-practitioner enrollments and 432,233 organizations across 321 provider types.



Medicare Advantage & Other Plans Now Cover the Majority

Medicare enrollment by coverage delivery system, May 2026 (Millions)



51.2%

MEDICARE ADVANTAGE & OTHER
SHARE
May 2026

What's happening

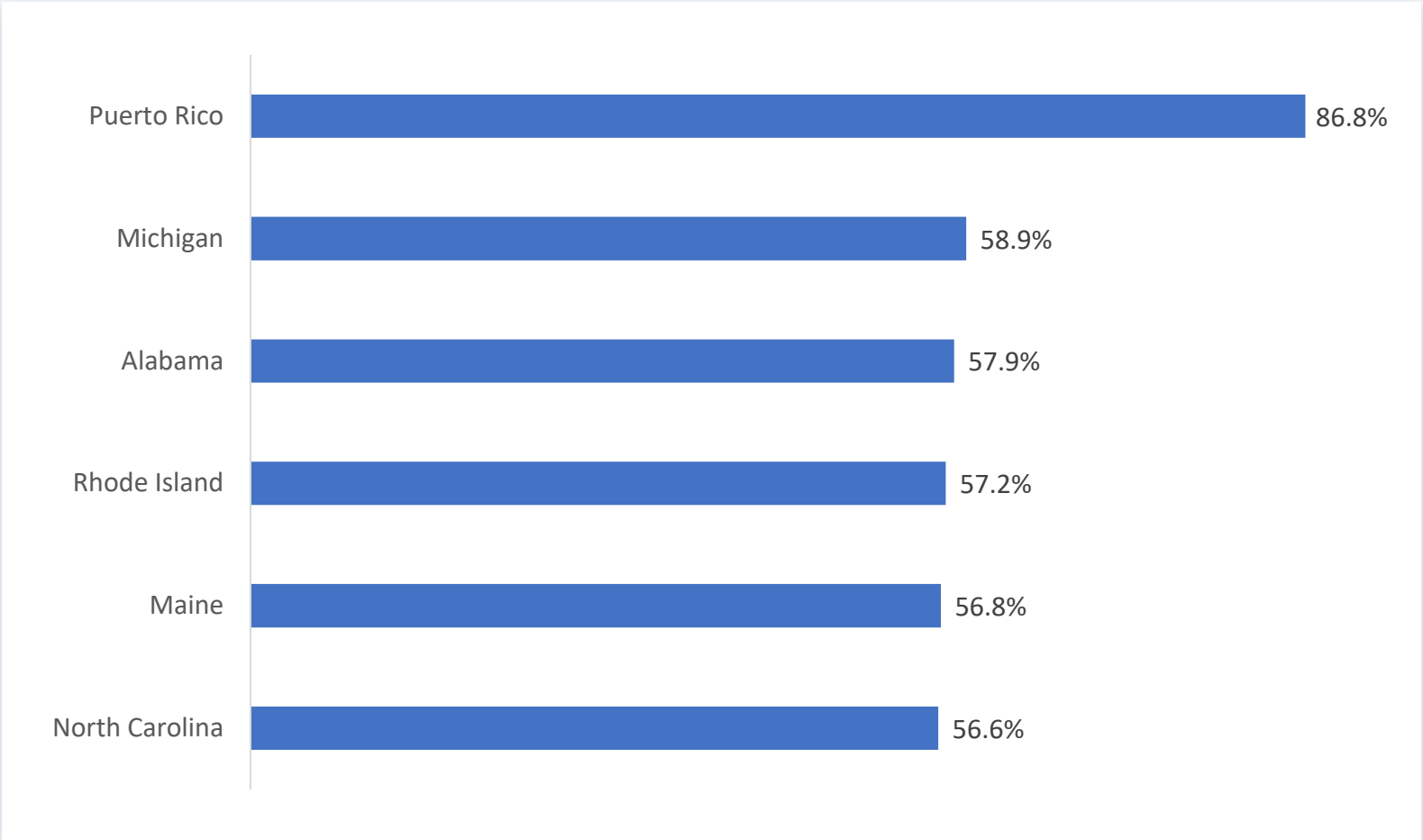
Of **70.5M** total beneficiaries, **36.1M** are in Medicare Advantage and other plans.

Medicare Advantage & Other now exceed Original Medicare by approximately 1.7M beneficiaries, reflecting a continuing shift in delivery-system mix.



Where Medicare Advantage Leads

Highest Medicare Advantage share by state or territory, May 2026.



86.8%

Highest Medicare Advantage Share

Puerto Rico

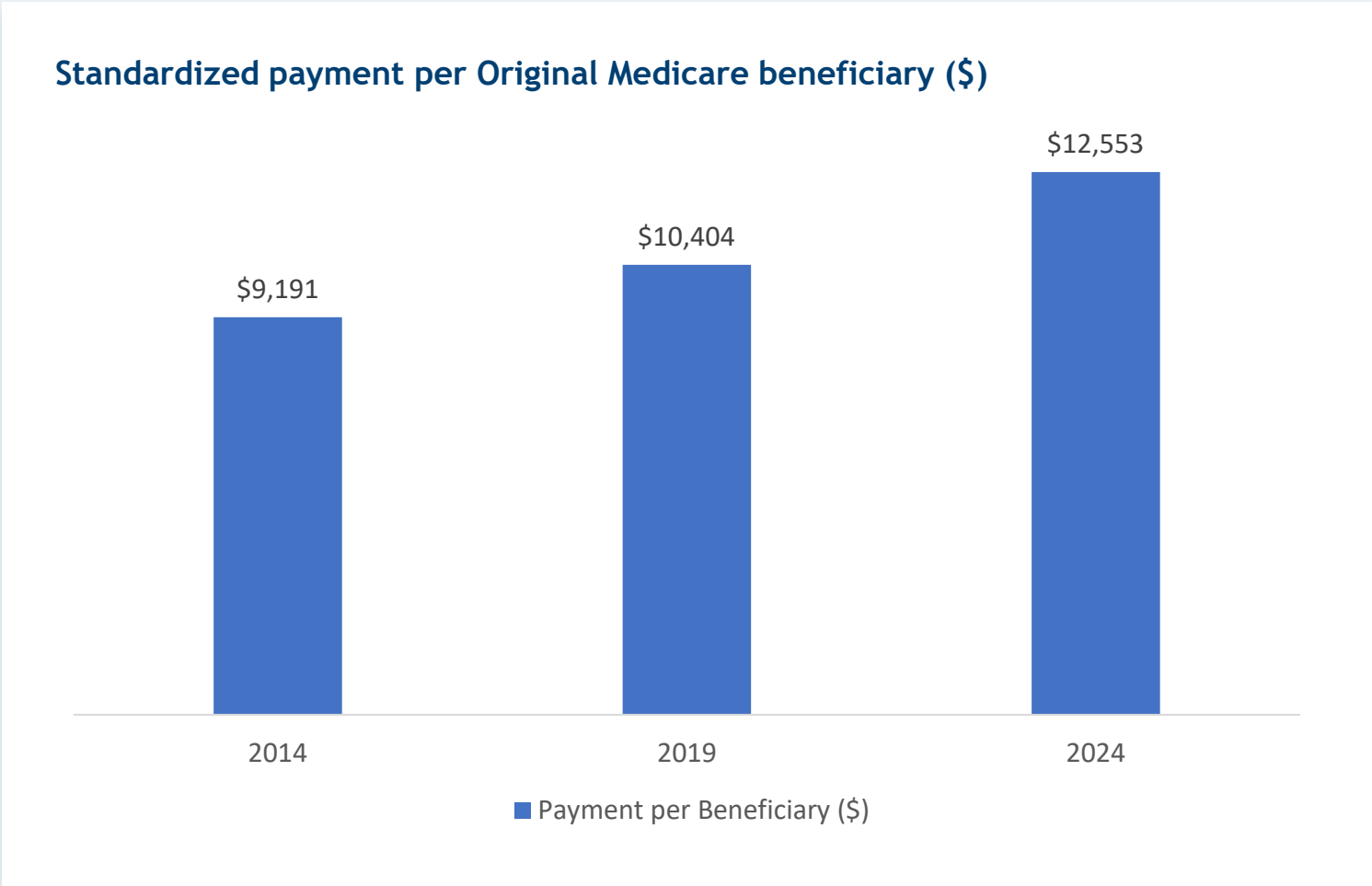
Concentration

Puerto Rico is a clear outlier at **86.8%**, well above every mainland state.

The mainland leaders cluster tightly in the high 50s — Michigan, Alabama, Rhode Island, Maine and North Carolina all sit near 57–59%, so MA majority is now ordinary rather than exceptional.



FFS Spending per Beneficiary Keeps Rising



\$377.3B

TOTAL FFS PAYMENTS
2024

What's happening

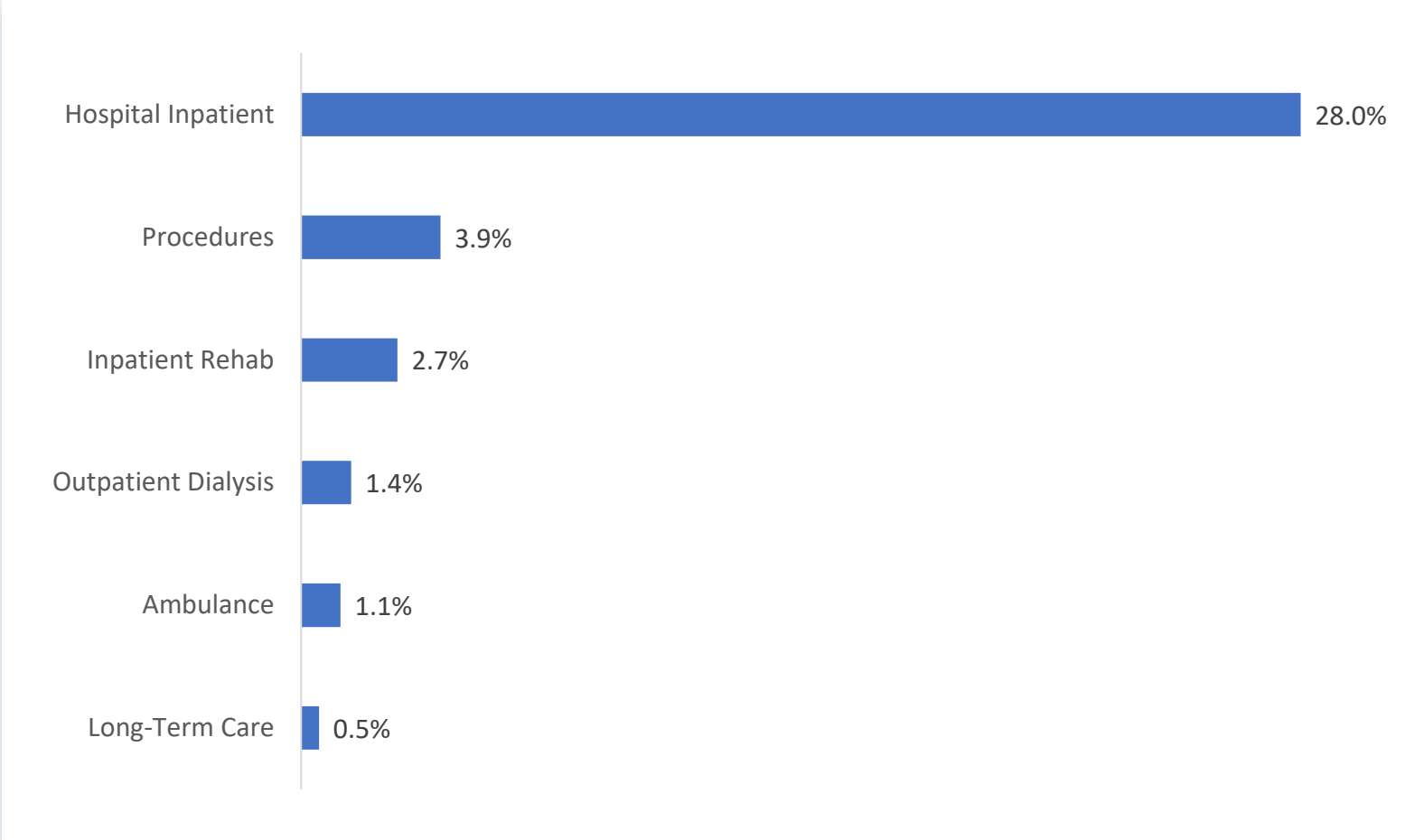
Standardized payment per beneficiary rose from **\$9,191** in 2014 to \$10,404 in 2019 and **\$12,553** in 2024.

That is a 37% increase per person over a decade, and it accelerated: +13% in the first five years, +21% in the second — on a shrinking Original Medicare population.



Where Fee-for-Service Dollars Go

Share of total Original Medicare FFS payments by service type, 2024 — selected published categories.



28.0%

LARGEST SERVICE TYPE

Hospital Inpatient

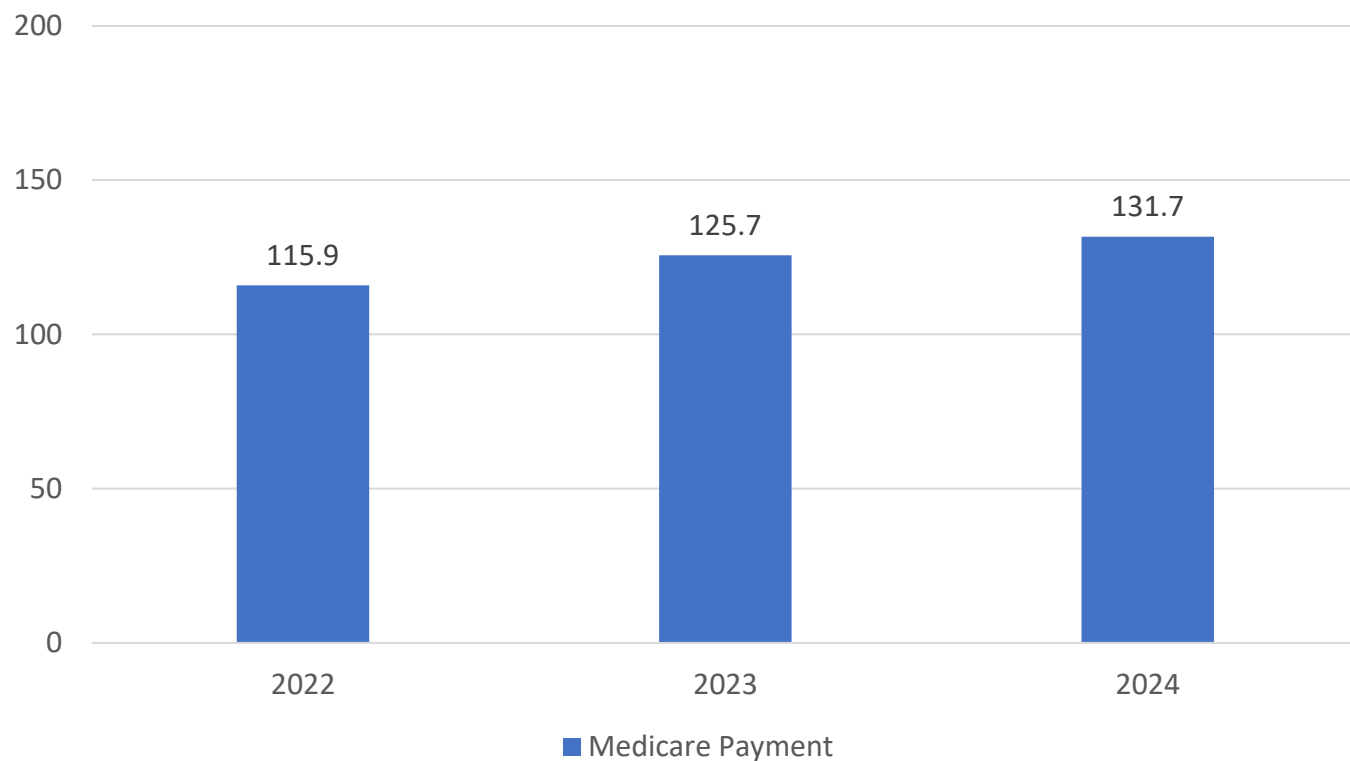
Concentration

Hospital inpatient alone absorbs 28.0% of every fee-for-service dollar.

Below hospital care the mix flattens into a long tail — procedures, rehab, dialysis and ambulance each land in the low single digits, so cost control has to start with the hospital block.

Medicare Part B Spending Is Climbing

Total Medicare Payment Amount by Year (\$ Billions)



+13.6%

2-YR PAYMENT GROWTH

2022 → 2024

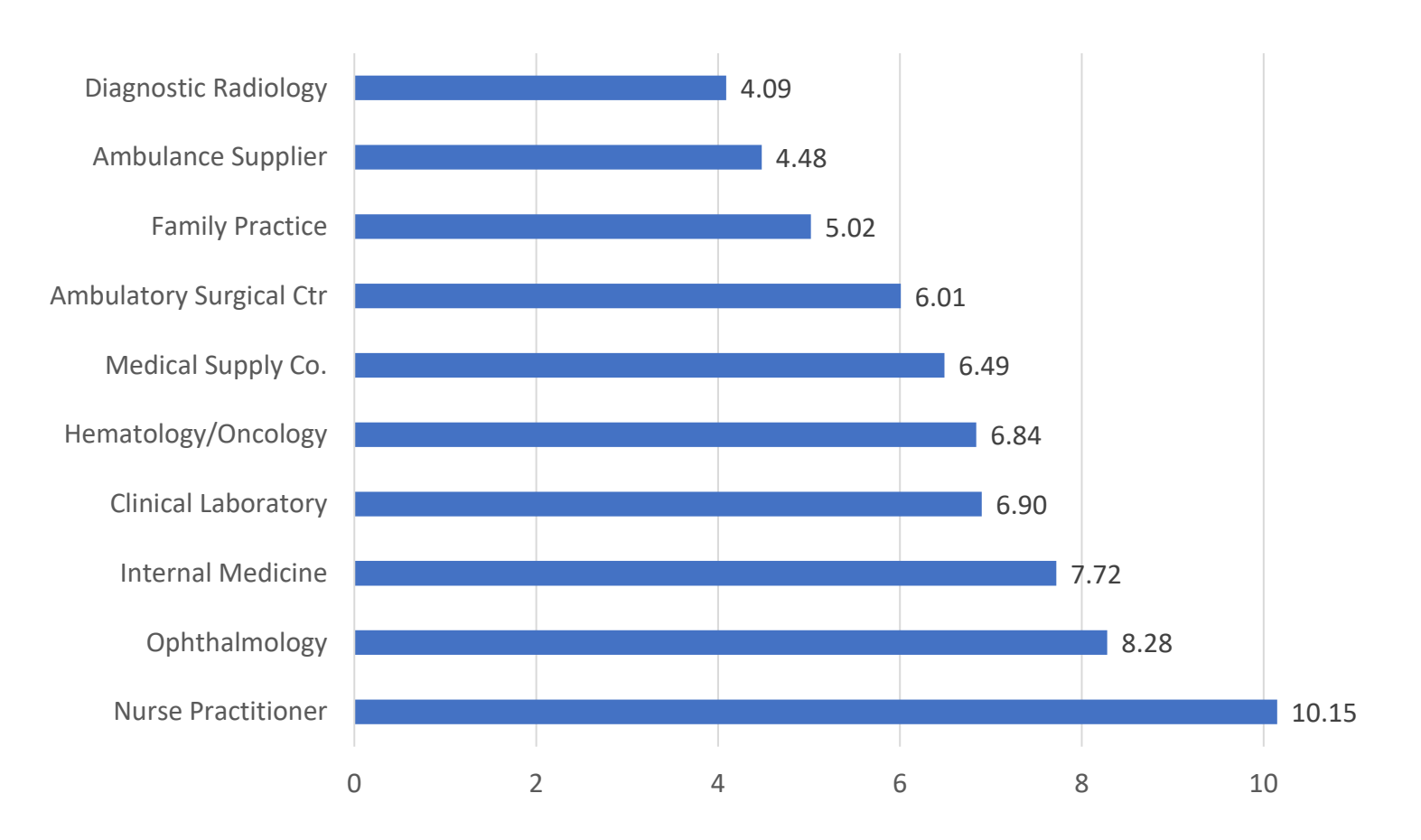
What's happening

Payments rose **+8.4%** in 2023, then a softer **+4.8%** in 2024.

Growth slowed in 2024; the technical appendix separates changes in service volume and payment per service.

Top 10 Specialties by Medicare Payment

Specialty payment totals, CY 2024 reporting year.



\$10.15B

TOP SPECIALTY

Nurse Practitioner

Concentration

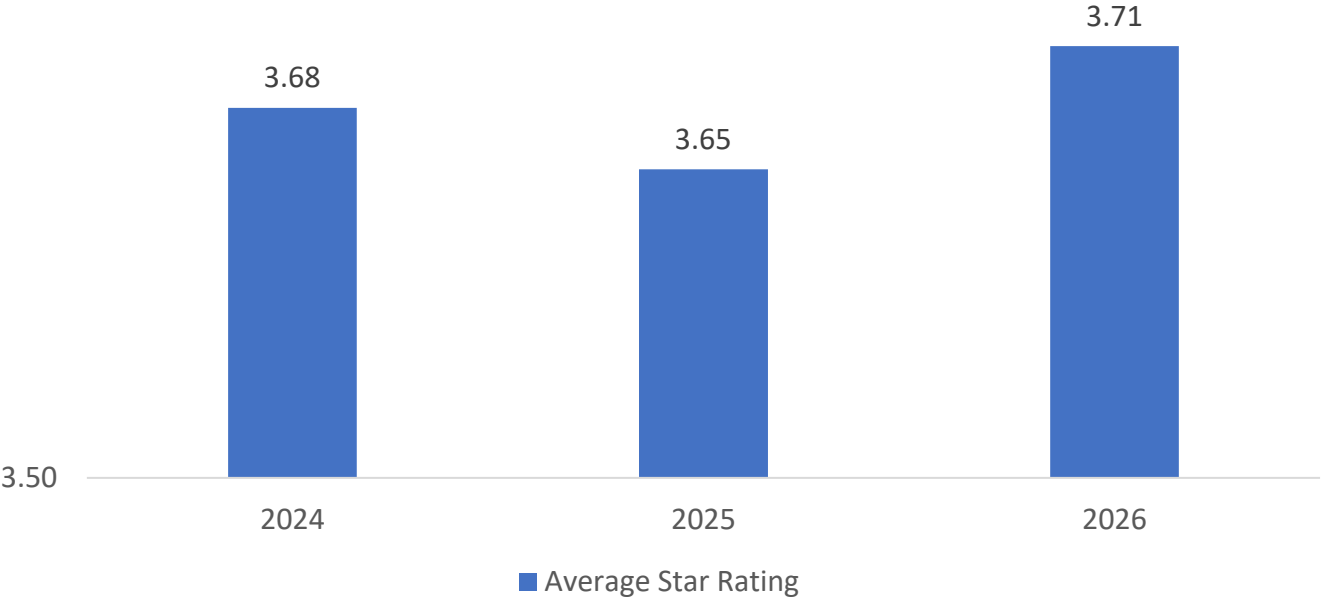
The top 10 specialties alone account for ~\$66.0B of 2024 payments.

Advanced-practice clinicians, ophthalmology, labs, and oncology anchor the spend — a mix of high-volume and drug-heavy fields.



Plan Quality Has Remained Stable Near 3.7 Stars

Average overall star rating, unweighted contract average, 2024-2026



3.7 / 5

AVG OVERALL RATING

516 rated contracts

What's happening

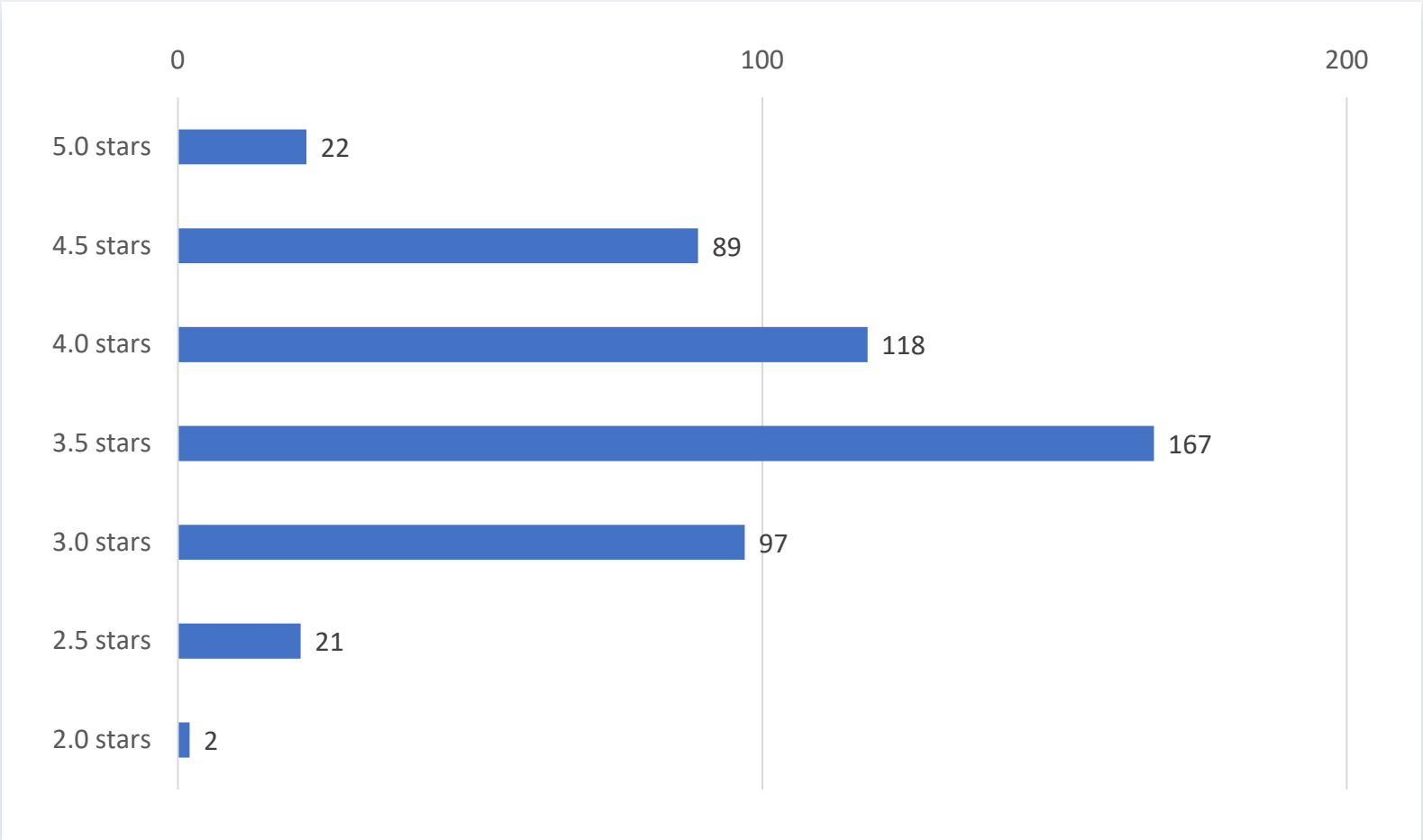
The average contract moved from **3.68** in 2024 to **3.71** in 2026 — effectively flat.

Average ratings have remained stable near 3.7 across three years, so gains are likelier to come from contract-level execution than from a rising tide.



Most Contracts Fall Short of Four Stars

Medicare Advantage contracts by CMS overall star rating, 2026 (n = 516).



Contract-level CMS overall star ratings; nonnumeric rating statuses excluded.

44.4%

CONTRACTS AT 4+ STARS

22 contracts at five stars

Implication

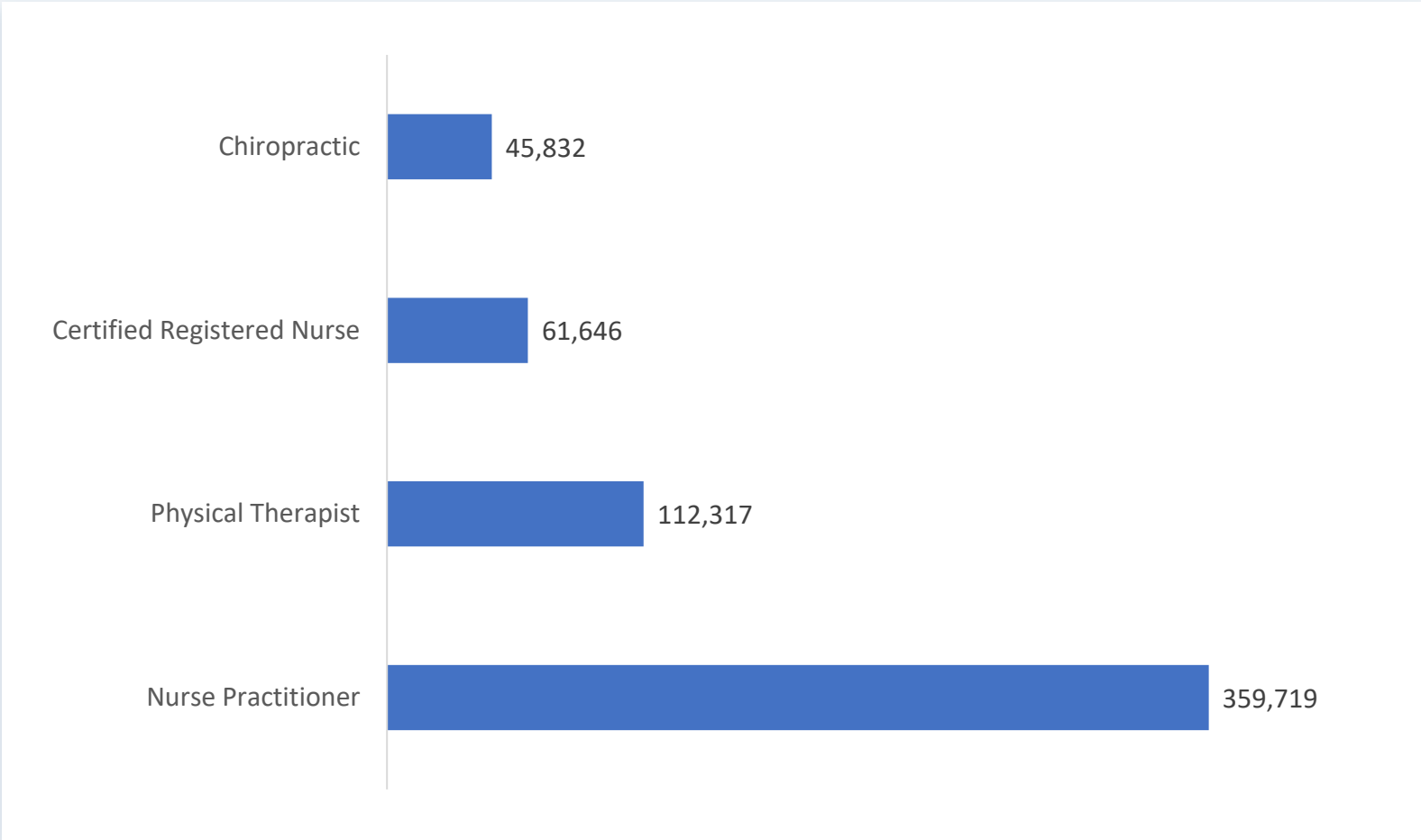
The distribution peaks at 3.5 stars — 167 contracts sit one half-step below the bonus threshold.

That cluster is a priority segment for contract-level measure review: focused measure improvements could move some contracts toward four stars.



Nurse Practitioners Are the Largest Primary Provider Type

Unique PECOS provider entities by reported primary provider type, 2026 Q3.



2.98M

ENROLLMENT RECORDS

PPEF 2026 Q3

Composition

Nurse practitioners are the largest primary provider type at **359,719** unique provider entities.

Across 321 primary provider types, 2.55M enrollments are individual practitioners and 432,233 are organizations and suppliers. This describes enrollment composition, not workforce supply or care delivered.

Where to Focus Next

01

Plan for an MA-majority program

With 51.2% of beneficiaries in plans, network adequacy, risk adjustment and star performance move more value than fee-for-service pricing levers.

02

Bend the per-beneficiary curve

Per-beneficiary spend rose 37% in a decade and is accelerating — +13% to 2019, then +21% to 2024. Hospital inpatient is the largest category at 28.0%.

03

Close the four-star gap

Only 44.4% of contracts reach 4+ stars and the average has been flat since 2024 — concentrate improvement on the 167 contracts sitting at 3.5.

04

Use enrollment mix in network analysis

Advanced-practice types dominate enrollment records; use that composition as an input to network design and analysis, not as a measure of care delivered.

05

Watch Part B unit cost, not just volume

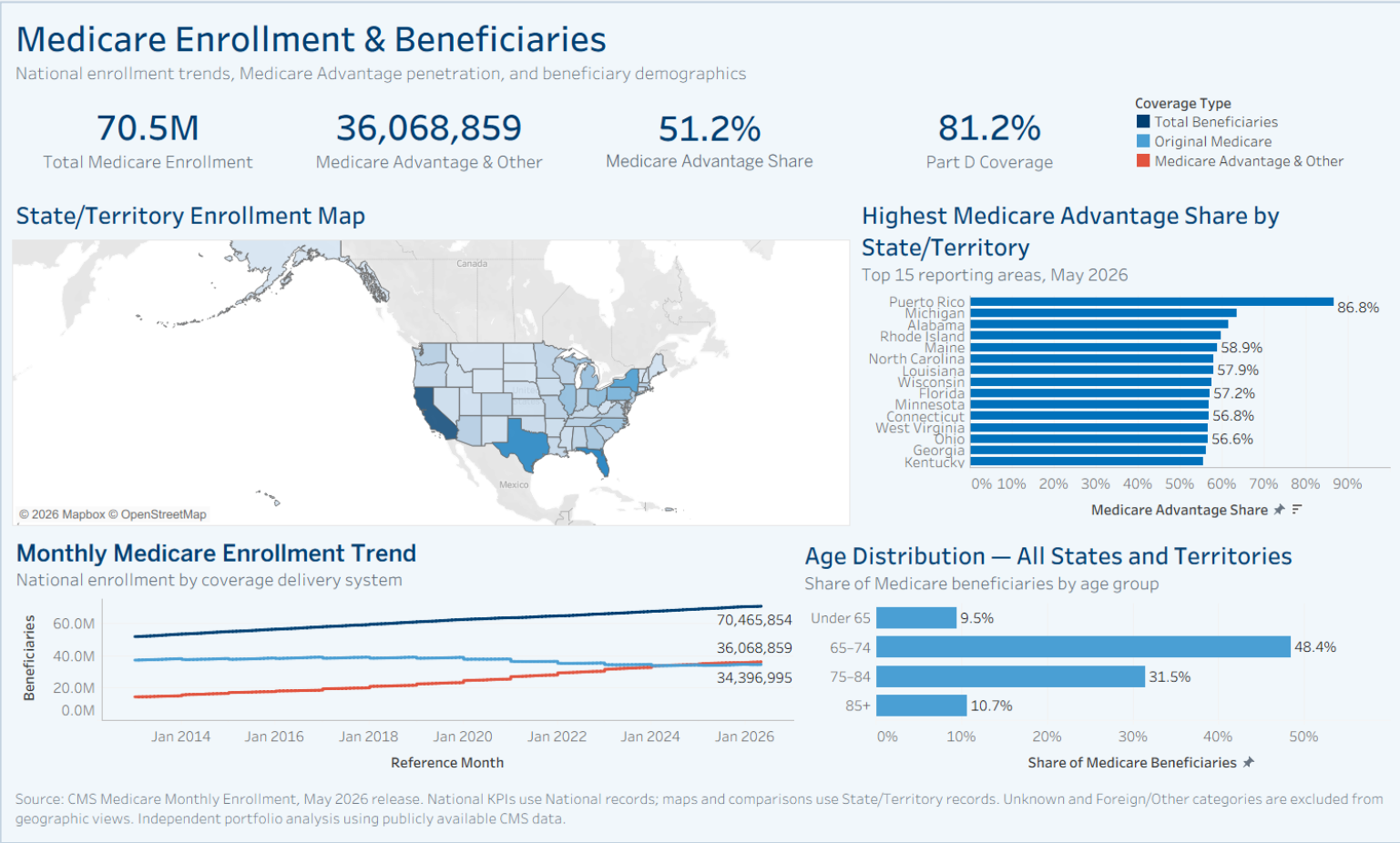
Part B payments rose 13.6% over two years while the payment rate held near 77% — track payment per service alongside volume.



Medicare Enrollment & Beneficiaries

Coverage mix, geography, and age structure.

Live on Tableau Public: public.tableau.com/app/profile/lisa.a.phillips

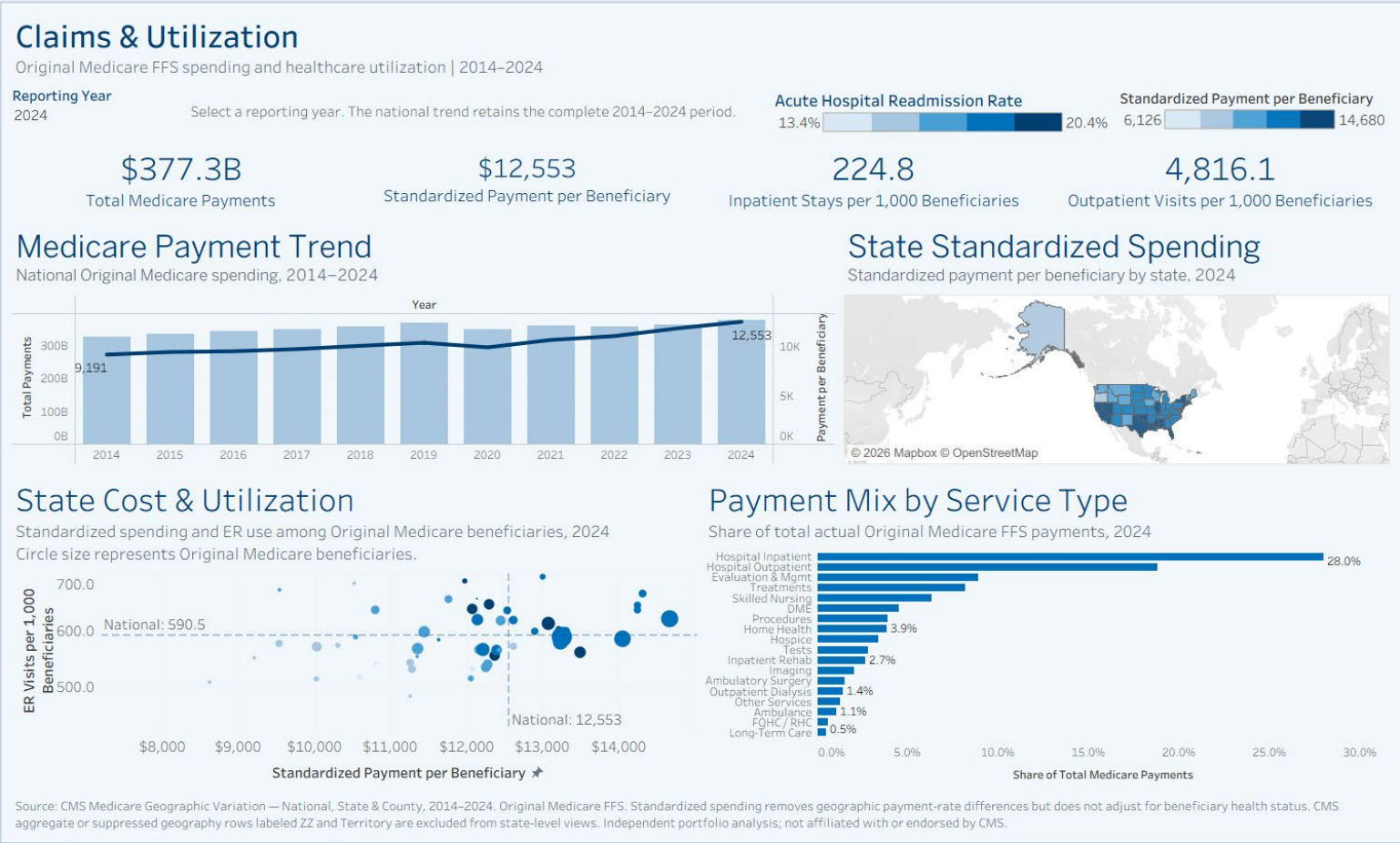




Claims & Utilization

FFS payments, service mix, and per-beneficiary cost.

Live on Tableau Public: public.tableau.com/app/profile/lisa.a.phillips

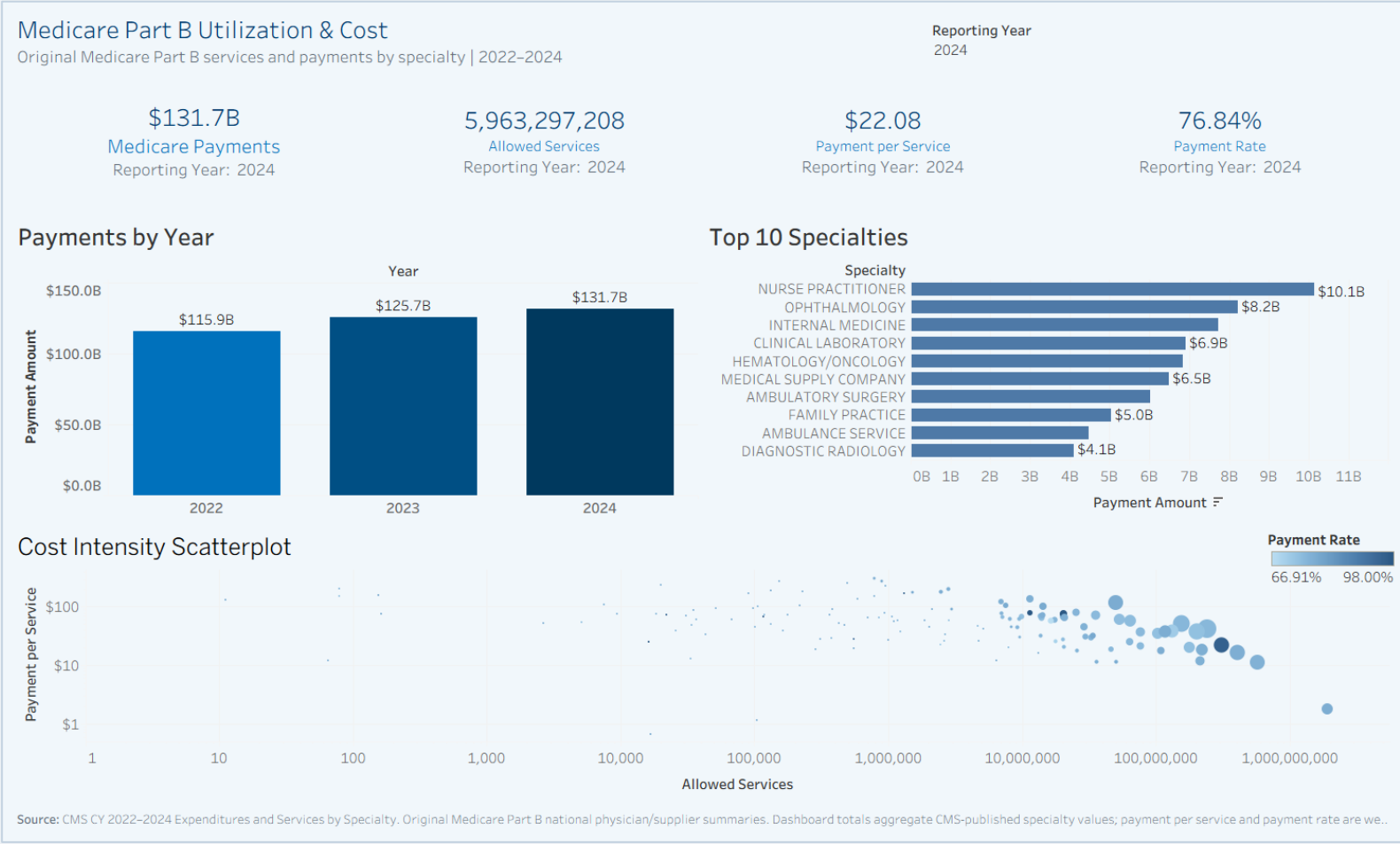




Medicare Part B Utilization & Cost

Services and payments by specialty, 2022–2024.

Live on Tableau Public: public.tableau.com/app/profile/lisa.a.phillips

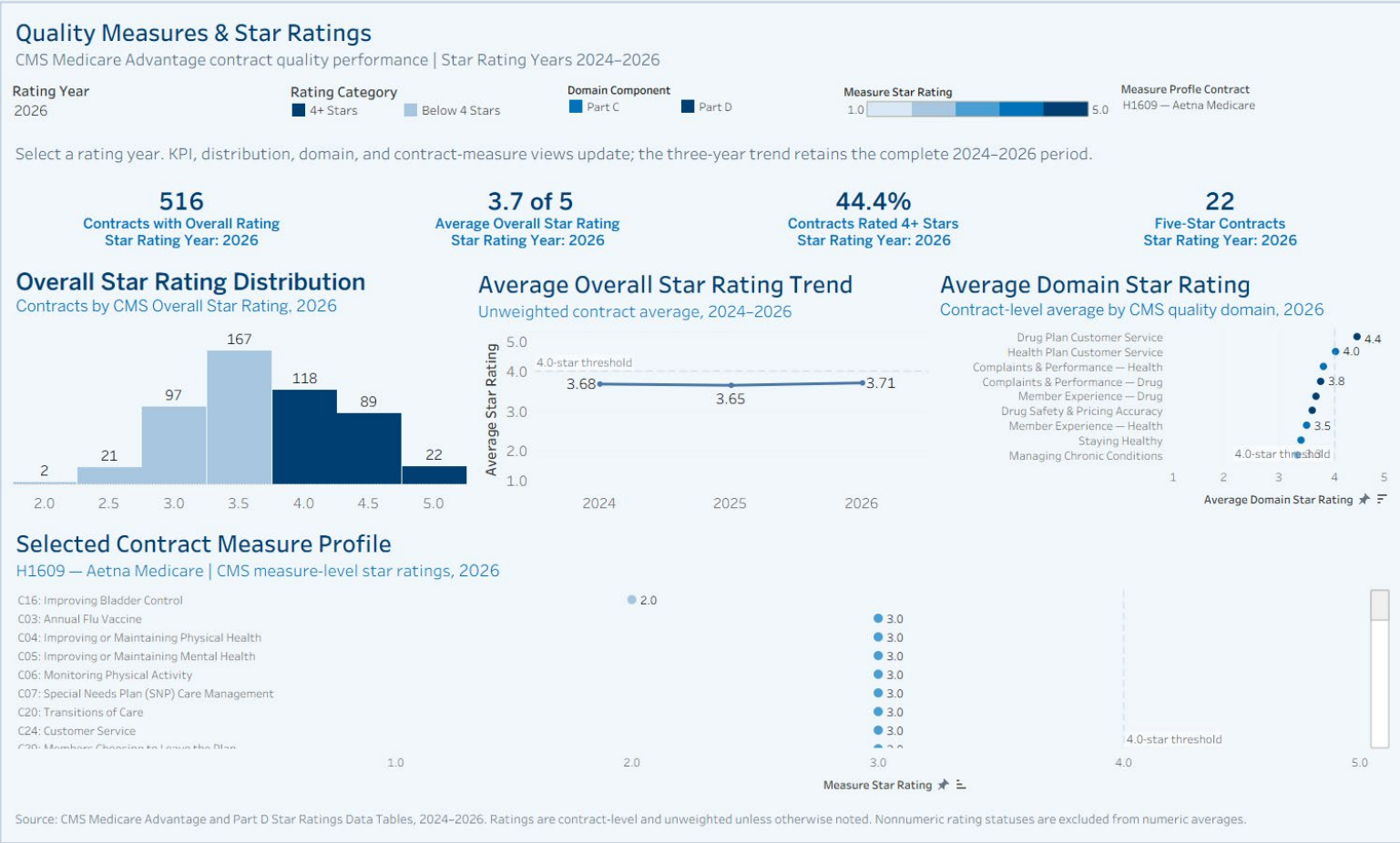




Quality Measures & Star Ratings

Contract-level rating trend and distribution.

Live on Tableau Public: public.tableau.com/app/profile/lisa.a.phillips

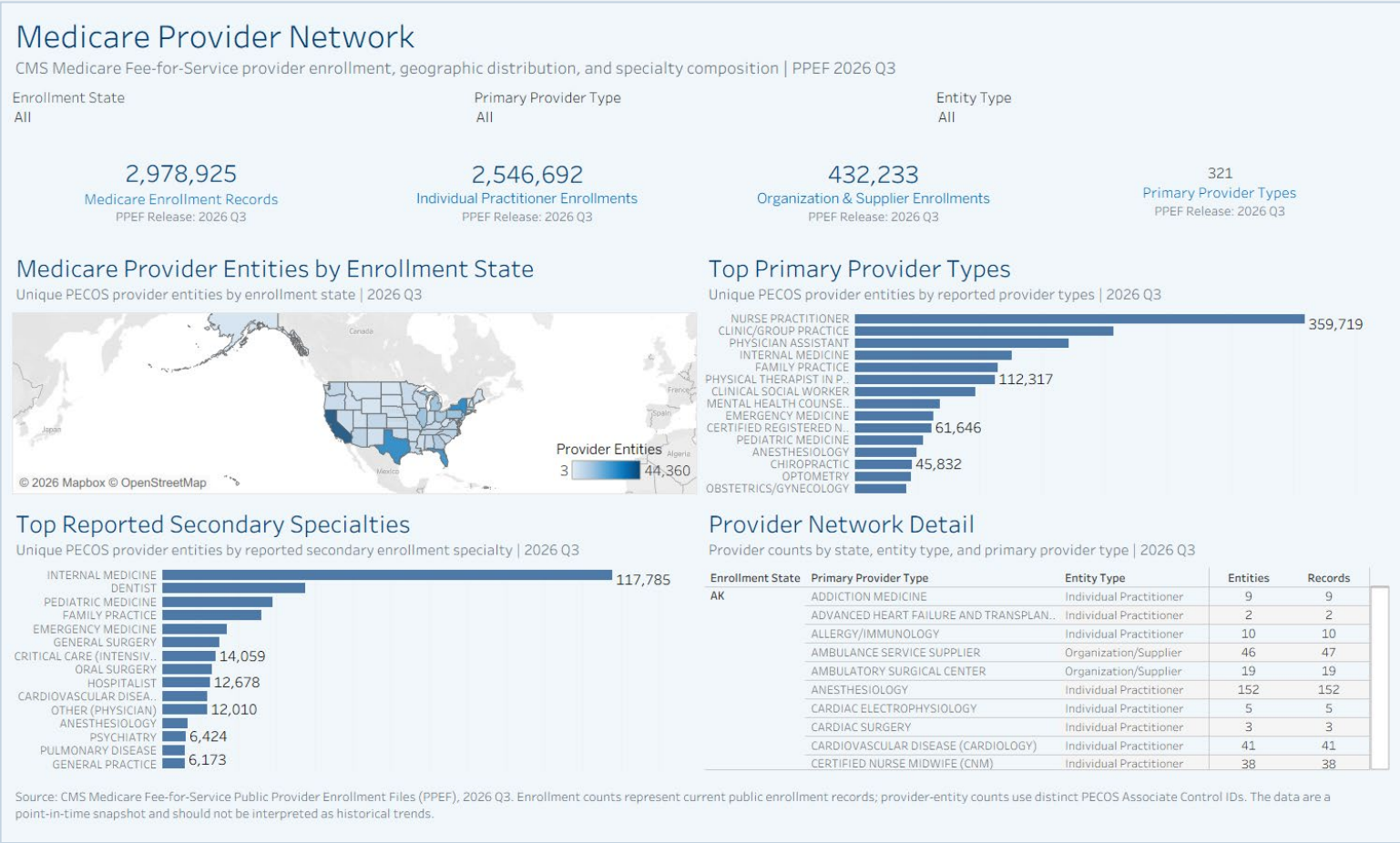




Medicare Provider Network

Enrollment composition by primary provider type.

Live on Tableau Public: public.tableau.com/app/profile/lisa.a.phillips



Sources & Notes

CMS Datasets

Enrollment

Medicare Monthly Enrollment — May 2026 release

<https://data.cms.gov/summary-statistics-on-beneficiary-enrollment>

Claims

Medicare Geographic Variation — National, State & County, 2014–2024

<https://data.cms.gov/medicare-geographic-variation>

Part B

Medicare Part B PBAR — National Summary, CY 2022–2024

data.cms.gov/provider-summary-by-type-of-service

Quality

MA and Part D Star Ratings Data Tables, 2024–2026

<https://www.cms.gov/medicare/health-plans/medicareadvtspecratestats>

Providers

Medicare FFS Public Provider Enrollment (PPEF), 2026 Q3

data.cms.gov/provider-characteristics/medicare-provider-supplier-enrollment

Notes & Limitations

- Aggregated public-use national summary data — not beneficiary-level claims.
- Provider figures use the PPEF enrollment extract related through ENRLMT_ID.
- Star ratings are contract-level; nonnumeric statuses excluded from averages.
- Part B specialty and national figures use the CY 2024 reporting year; trend spans CY 2022–2024.
- Aggregate/Total and source rows excluded; figures rounded for presentation.
- Independent portfolio analysis; not commissioned, reviewed, endorsed, or published by CMS.

APPENDIX

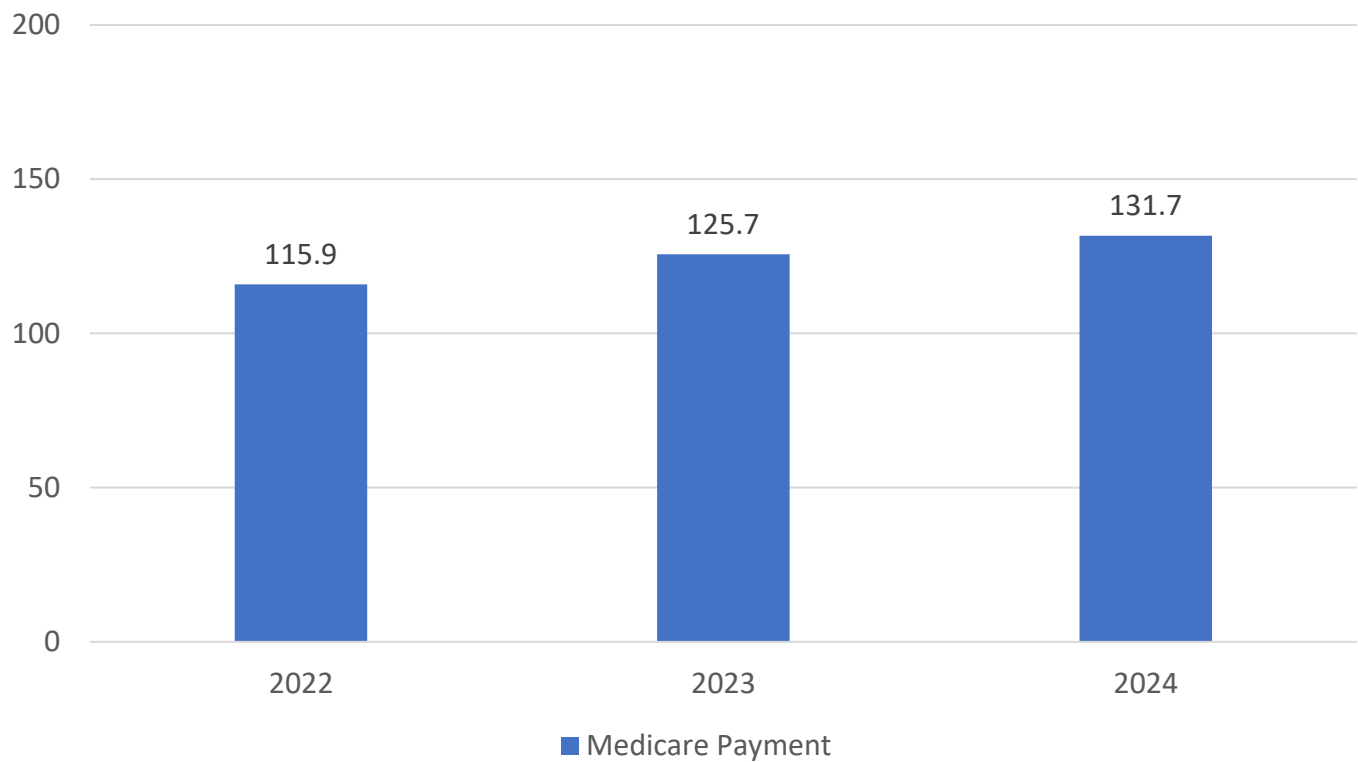
Technical Appendix: Part B Specialty Analysis

Descriptive Part B analysis, analyst-defined specialty segments, and the payment reconciliation checks that support the main findings.



Medicare Part B Spending Is Climbing

Total Medicare Payment Amount by Year (\$ Billions)



+13.6%

2-YR PAYMENT GROWTH

2022 → 2024

What's happening

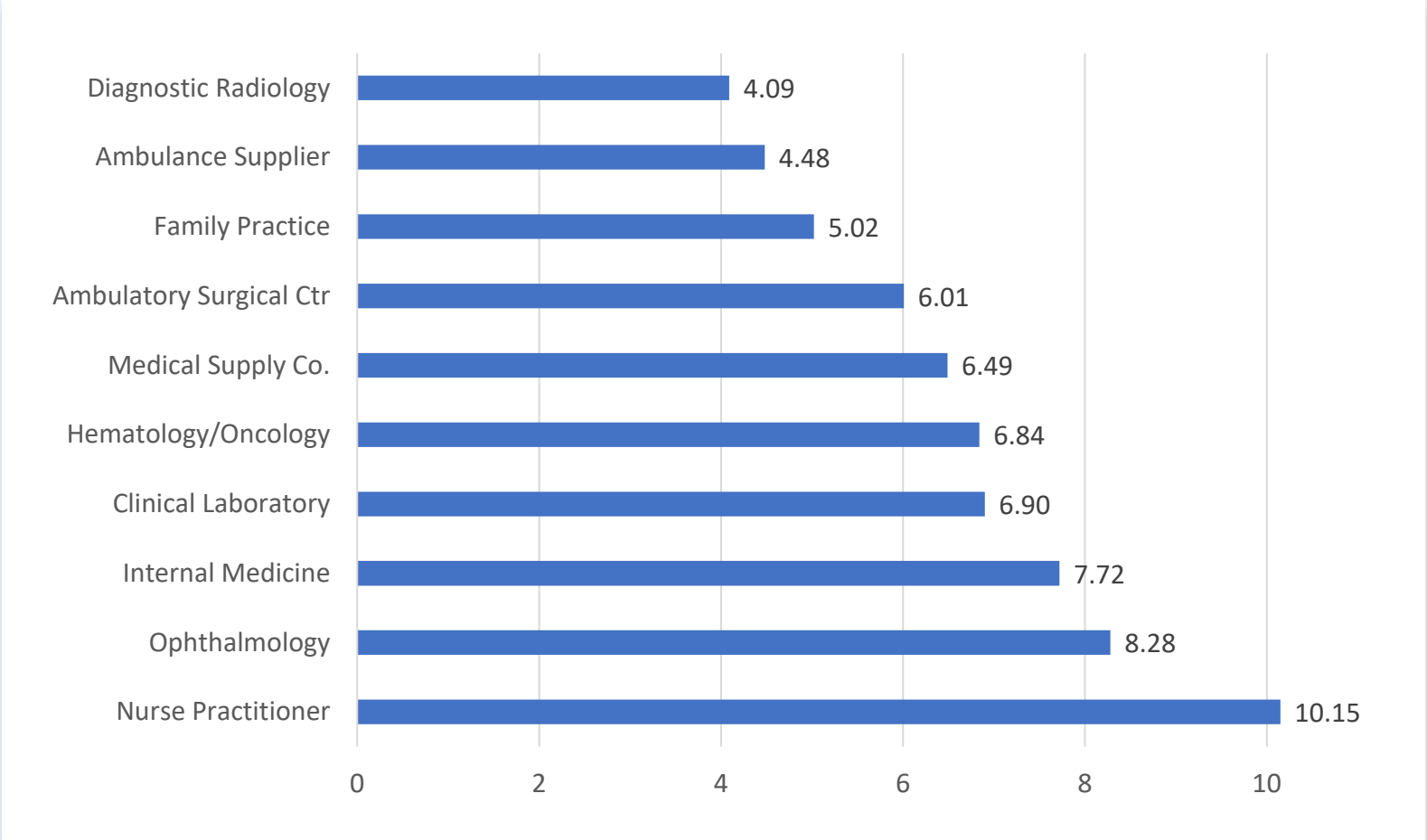
Payments rose **+8.4%** in 2023, then a softer **+4.8%** in 2024.

Growth slowed in 2024; the following analysis separates changes in service volume and payment per service.



Top 10 Specialties by Medicare Payment

Specialty payment totals, CY 2024 reporting year.



\$10.15B

TOP SPECIALTY

Nurse Practitioner

Concentration

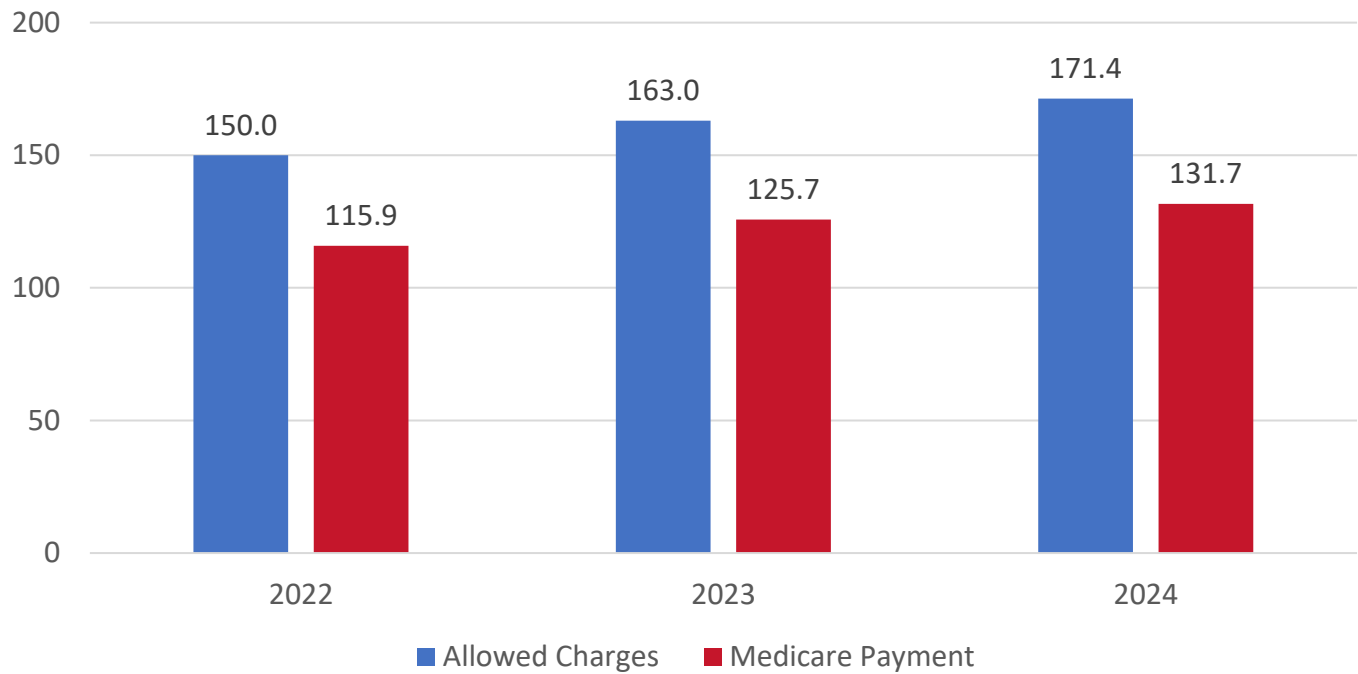
The top 10 specialties alone account for ~\$66.0B of 2024 payments.

Advanced-practice clinicians, ophthalmology, labs, and oncology anchor the spend — a mix of high-volume and drug-heavy fields.



Payment Rate Holds Near 77%

Allowed Charges vs. Medicare Payment (\$ Billions)



Payment rate: 2022 77.3% 2023 77.1% 2024 76.8%

76.8%

2024 PAYMENT RATE
of allowed charges paid

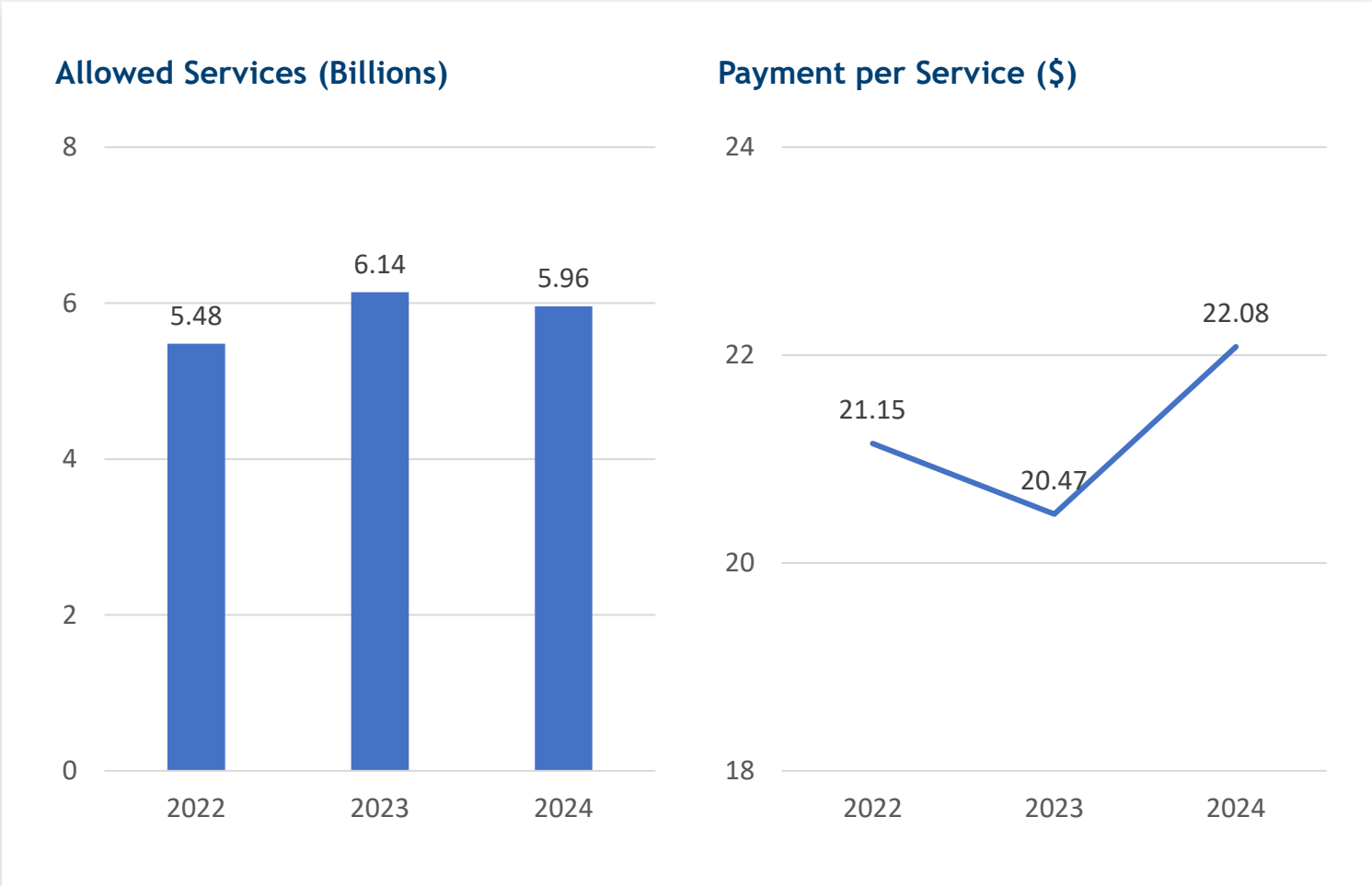
Why it matters

Roughly **77 cents on every approved dollar** is paid, and that ratio barely moves year to year.

The ratio eased slightly to 76.8% in 2024 — a change small enough that no single driver can be identified from these data.



Volume Peaked in 2023; Unit Cost Rebounded



6.14B

PEAK SERVICES (2023)

eased to 5.96B in 2024

Reading the split

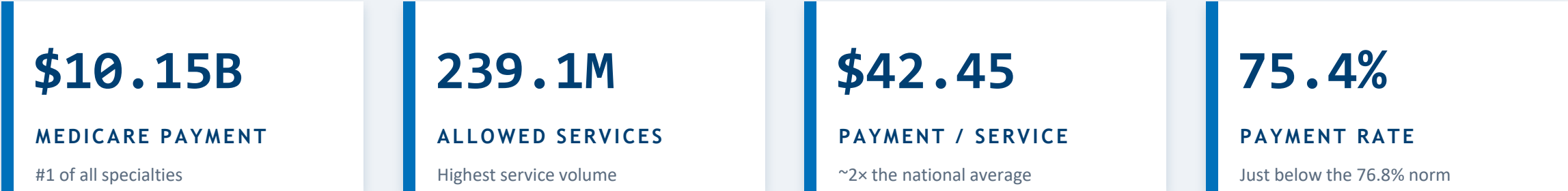
Service volume **peaked in 2023** then eased, while payment per service **dipped then rebounded to \$22.08**.

Both components moved in 2024: service volume eased while payment per service rose.



Nurse Practitioner: The New Volume Leader

The largest specialty by Medicare payment in CY 2024 — advanced-practice volume at a mid-range unit price.



Why it tops the ranking

Nurse practitioners bill **239 million services** a year at **\$42.45 each** — advanced-practice volume has grown fast enough to overtake Internal Medicine on total payment.

On **\$13.46B** of allowed charges, Medicare pays 75.4% — slightly below the national norm. Rising service volume, not unit price, drives the rank.

How it compares

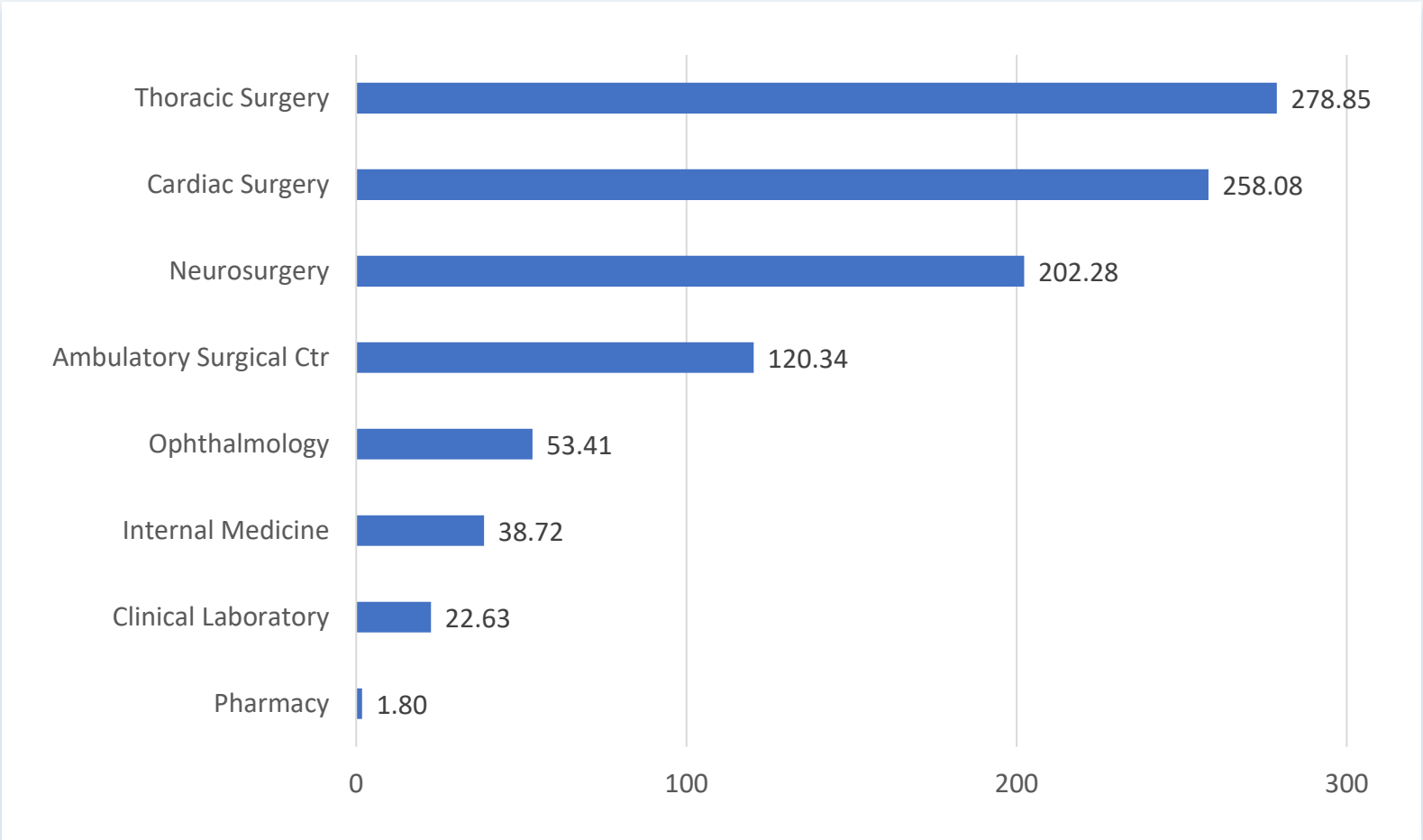
Nurse practitioners bill at roughly twice the national payment per service, on immense volume.

	Nurse Practitioner	All Part B
Allowed services	239.1M	5.96B
Payment / service	\$42.45	\$22.08
Payment rate	75.4%	76.8%



A 155× Spread in Payment per Service

Selected specialties, CY 2024 payment per service (\$) — commodity dispensing to complex surgery.



Specialty figures are CY 2024; national trends span CY 2022–2024.

155×

COST INTENSITY SPREAD

\$1.80 → \$279 / service

Implication

Total spend and unit cost tell different stories.

Commodity fields win on volume; surgical fields win on intensity. Any cost-management lever must be matched to the right group.



Three Analytical Specialty Segments

Analyst-defined clusters: K-means on log-transformed, standardized volume, payment, payment-per-service and payment rate; k = 3 chosen by elbow and interpretability.

01 High-Volume Commodity

LABS, PHARMACY, IMAGING, SUPPLIES

Low unit cost, immense throughput, high payment rate. Managed through pricing and volume controls.

02 Balanced High-Spend

INTERNAL MEDICINE, FAMILY PRACTICE, NP, OPHTHALMOLOGY

Moderate unit cost at large scale — the core of aggregate spending.

03 Specialized High-Intensity

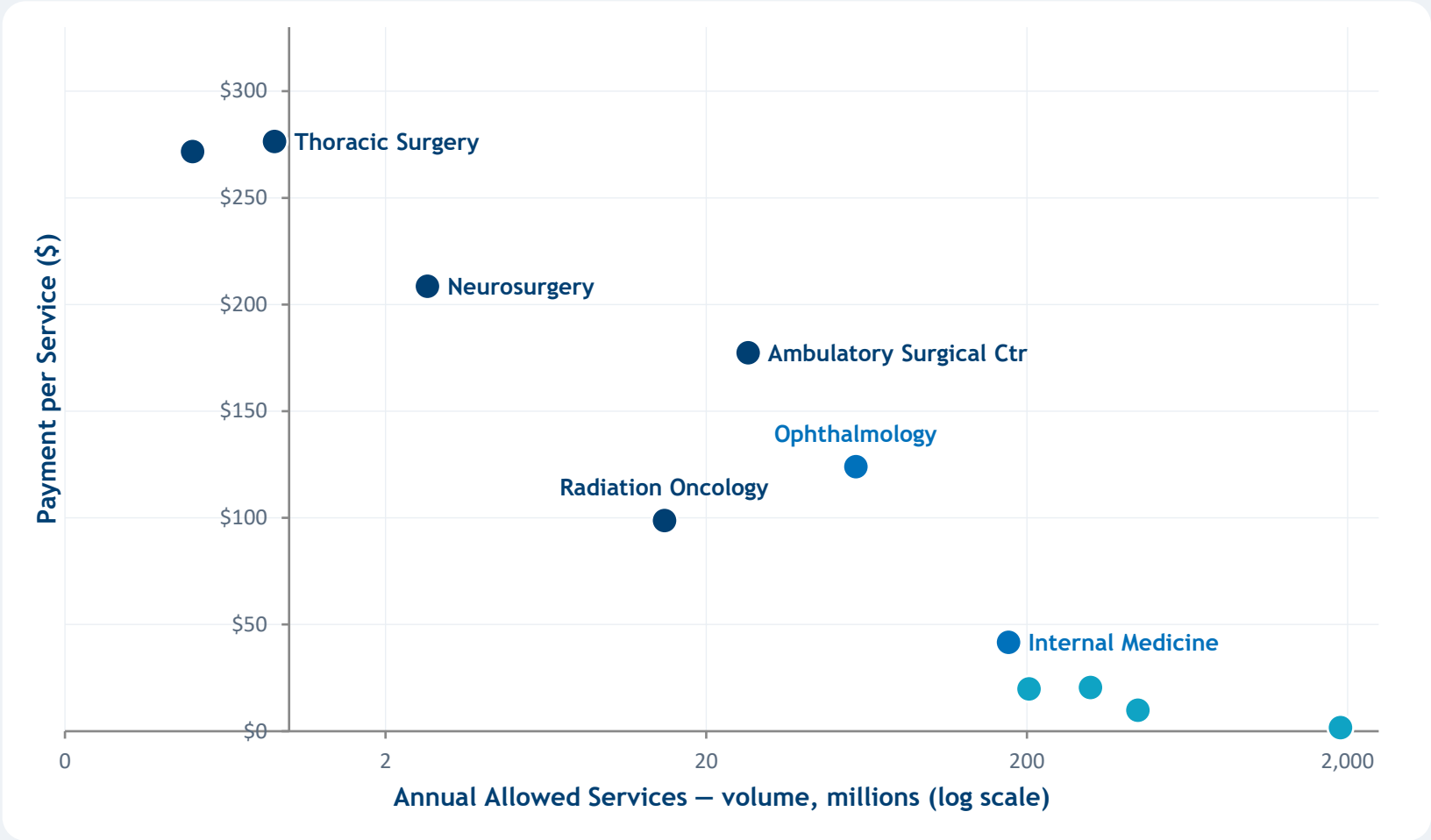
CARDIAC, THORACIC & NEUROSURGERY, ONCOLOGY

Low volume, very high payment per service. Managed through appropriateness and clinical review.



The Clusters, Made Visible

Allowed services vs. payment per service, 12 representative specialties by analyst-defined cluster.



READING THE CHART

Each dot is a specialty, positioned by annual volume and unit cost.

High-Volume Commodity
Low unit cost, immense volume

Balanced High-Spend
Moderate cost at large scale

Specialized High-Intensity
High cost, low volume

Clusters separate cleanly. Volume and unit cost trade off along a clear diagonal. Unlabelled lower-left points: Pharmacy, Medical Supply, Clinical Lab, Diagnostic Radiology.

Tableau Build Blueprint: Part B Specialty Analysis

A step-by-step recipe to reproduce this dashboard — data source, worksheets, and the assembled canvas.

1 · DATA SOURCE & FIELDS

- Connect CMS PBAR national summary files (CY 2022–2024).
- Union the three yearly files into one specialty-year table.

Dimensions: Specialty, Year

Measures: Allowed Services, Allowed Charges, Payment

Calc — Pay/Svc: $SUM([Payment]) / SUM([Services])$

Calc — Rate: $SUM([Payment]) / SUM([Allowed Charges])$

- Exclude aggregate / “All” / source rows.

2 · SHEET: KPI TILES

Marks: Text (BAN)

Text: Payment, Services, Pay/Svc, Rate

Layout: one BAN per KPI, 4 across

3 · SHEET: PAYMENTS BY YEAR

Marks: Bar

Columns: YEAR(Year)

Rows: $SUM([Payment])$

Label: $SUM([Payment])$; blue fill

4 · SHEET: TOP SPECIALTIES

Marks: Bar (horizontal)

Rows: Specialty (sort desc)

Columns: $SUM([Payment])$

Filter: Top 10 by Payment

5 · SHEET: COST INTENSITY

Marks: Circle (scatter)

Columns: $SUM([Services])$

Rows: Payment / Service

Detail / Size: Specialty / Payment

6 · DASHBOARD ASSEMBLY

LAYOUT

Tiled container: KPI row on top, trend + top-specialties in the middle band, cost-intensity scatter below.

FILTERS & PARAMETERS

Year & Specialty as dropdowns, applied to all worksheets on the dashboard.

ACTIONS

Click a specialty bar to filter the trend and scatter (dashboard filter action) for instant drill-down.



Payment Reconciles to Volume × Price

FITTED MODEL

$$\text{Payment} = \text{Services} \times \text{Allowed Charge} / \text{Service} \times \text{Payment Rate}$$

Payment is volume × price × payment rate by construction, so the near-perfect fit confirms the accounting reconciles.

MODEL FIT

$$R^2 = 0.9995$$

Share of payment variance explained

$$\text{Test } R^2 = 1.00$$

70/30 holdout validation

$$\text{Error} = 1.9\%$$

Median payment miss, all 125

An identity check, not a predictive finding — OLS on all 125 CY 2022 specialties.

What this actually tells us

Consistent with the payment identity

Both elasticities ≈ 1.0, as expected

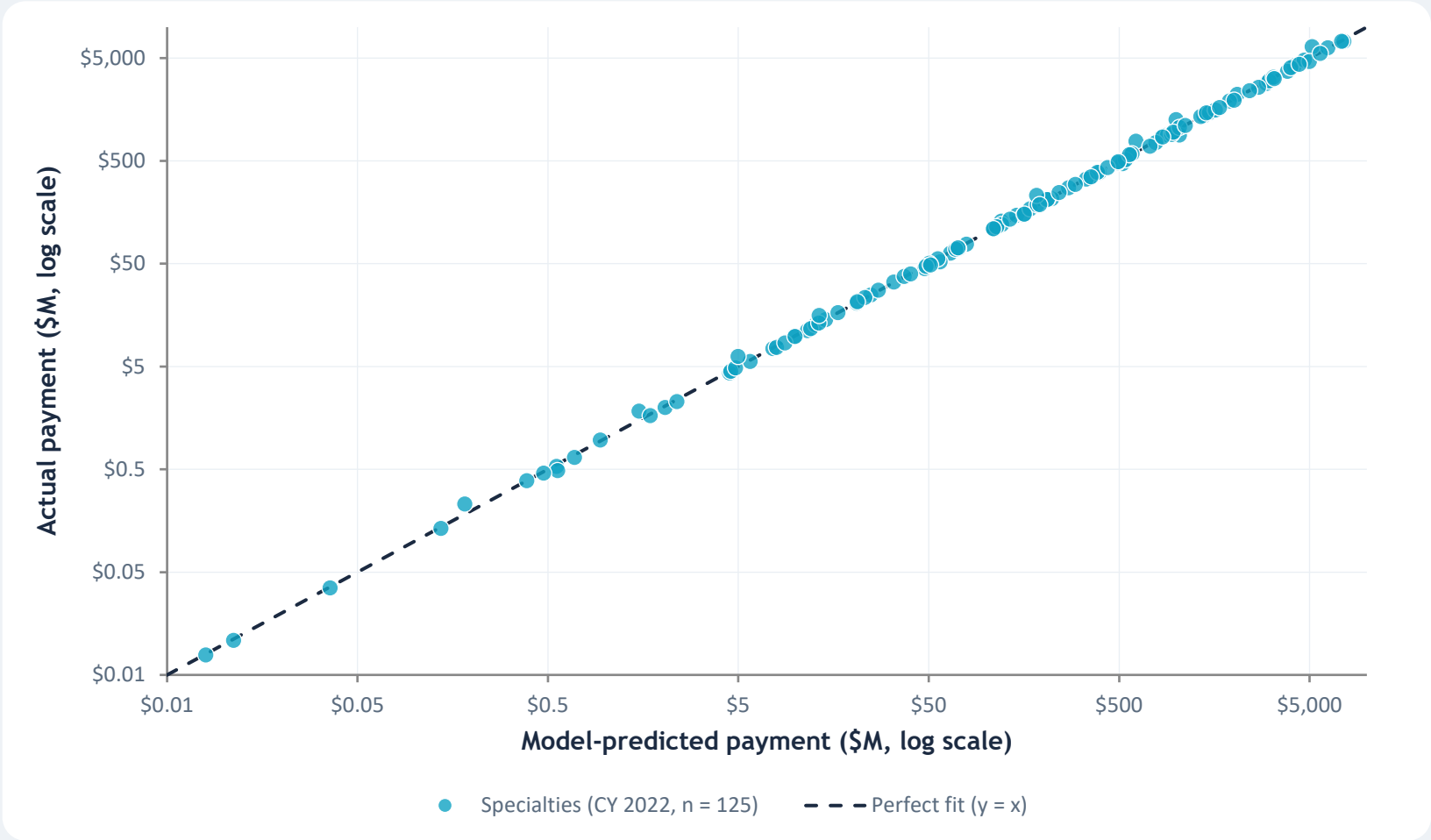
Payment rate near-constant (~78%)

Checked on all 125 CY 2022 specialties



The Fit Holds, Year After Year

Predicted vs. actual payment, all 125 CY 2022 specialties — 2023 and 2024 refit reproduce it.



DOES IT HOLD? YES

Refit each year, the model barely moves.

YEAR	MODEL R ²	VOL. ELAST.	PAY RATE
2022	0.9995	1.00	77.3%
2023	0.9996	1.00	77.1%
2024	0.9997	1.00	76.8%

The fit is structural. Payment $\approx$ volume $\times$ price $\times$ a near-constant rate — so the relationship reproduces every year, not just 2022.



Six Core Measures

Every specialty-year record is measured on the same six dimensions of volume, cost, and payment.

Allowed Services

Count of covered services delivered — the core utilization volume measure.

Allowed Charges

Medicare-approved billed amount across all allowed services.

Payment Amount

Dollars actually paid by Medicare — the spending headline.

Charge / Service

Allowed charges ÷ services — average approved price per service.

Payment / Service

Payment ÷ services — average dollars paid per service (cost intensity).

Payment Rate

Payment ÷ allowed charges — share of approved charges that is paid.



Analytical Approach

Data Preparation

- 1 Combine years**
Stack CY 2022–2024 into one specialty-year table.
- 2 Standardize names**
Normalize specialty labels for consistent grouping.
- 3 Exclude Total row**
Remove aggregate/source rows from specialty analysis.
- 4 Validate numerics**
Parse currency, services, and rate fields; check ranges.

Analysis Tracks

Descriptive

Totals, averages, medians, percent change, correlations, and top/bottom rankings.

Reconciliation & Validation

Regression used as an accounting-identity check on payment amount; evaluated with MAE, RMSE, and R^2 .

Segmentation

K-means ($k = 3$, chosen by elbow) on log-transformed, standardized volume and cost measures; surfaces high-volume, high-cost and specialized low-volume clusters.